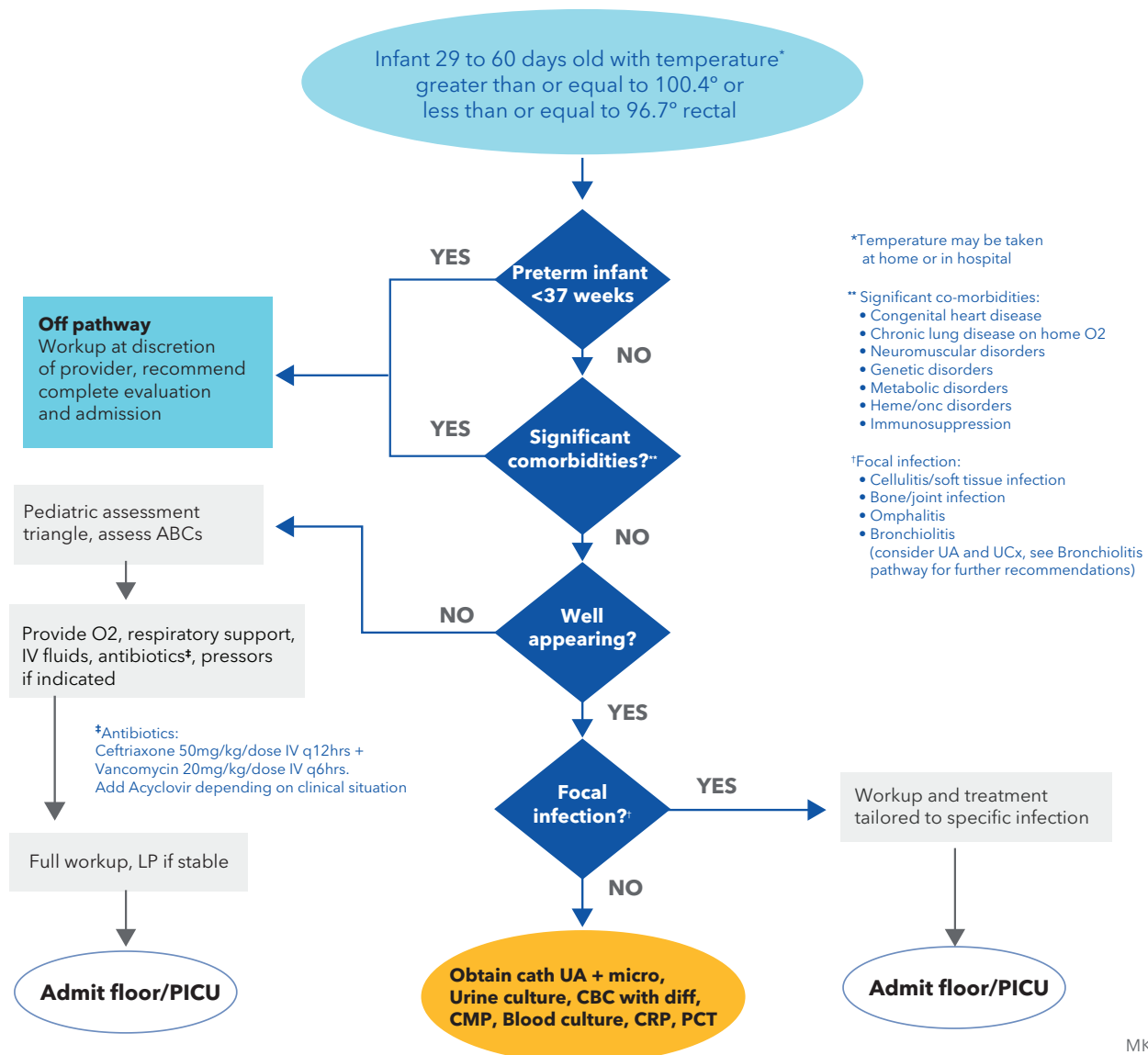


INFANT FEVER/ HYPOTHERMIA PROTOCOL (29-60 DAYS OLD)

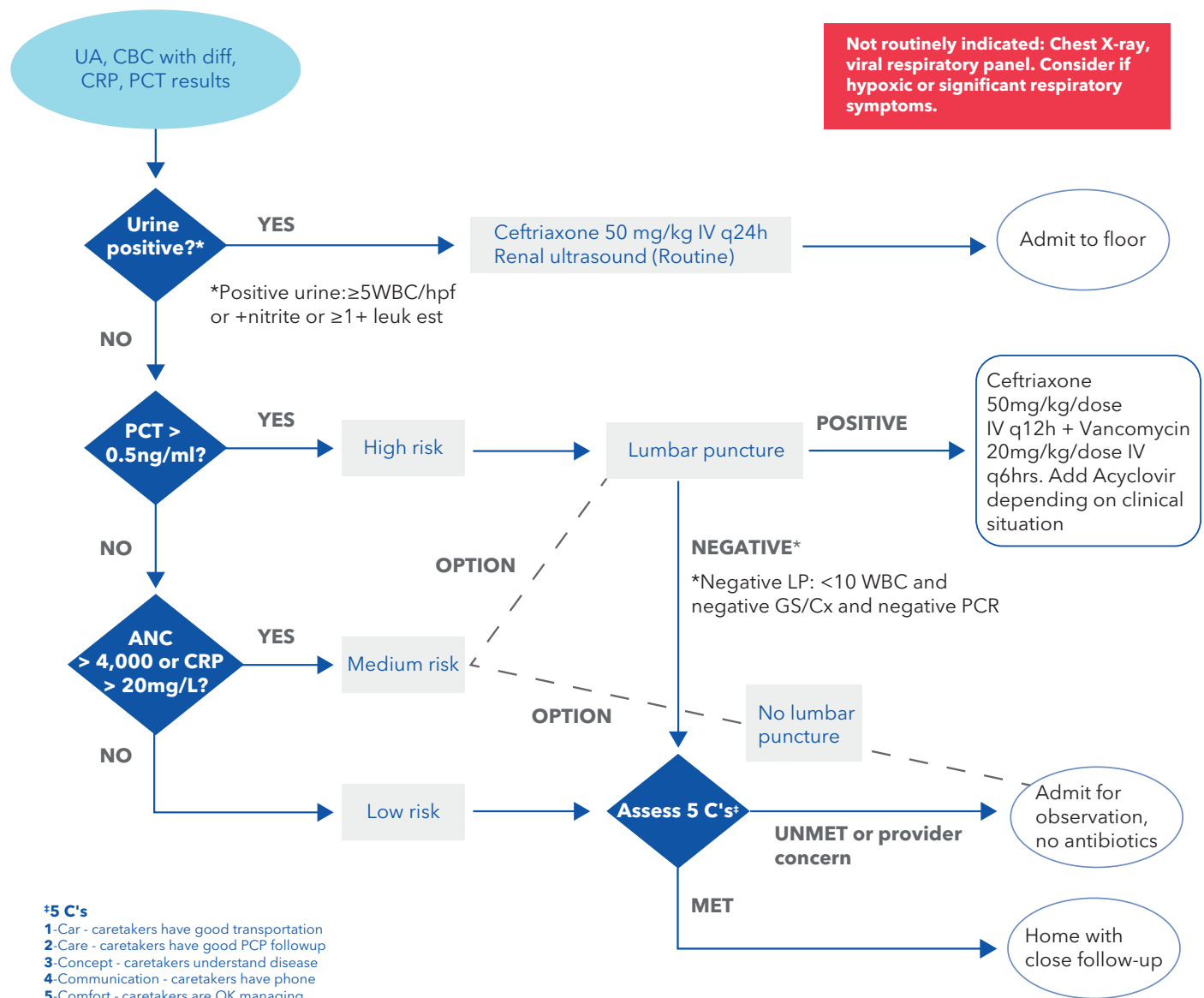
Goal: To identify (risk stratify) patients at low risk of IBI or SBI (Invasive or Serious bacterial infection) where observation or outpatient management may be suitable.

This clinical pathway outlines an evidence-based approach to the work-up and treatment of *well-appearing* infants aged 29–60 days with fever or hypothermia. Once inclusion and exclusion criteria are met, patients are risk-stratified according to laboratory parameters. These best practice guidelines are based on current literature (most notably PECARN and Step-by-Step pathways) and consensus of University of Kentucky pediatric hospitalists, pediatric emergency medicine, pediatric pharmacy, and emergency medicine pharmacy.

Please refer to the references at the end of the pathway for in-depth information.

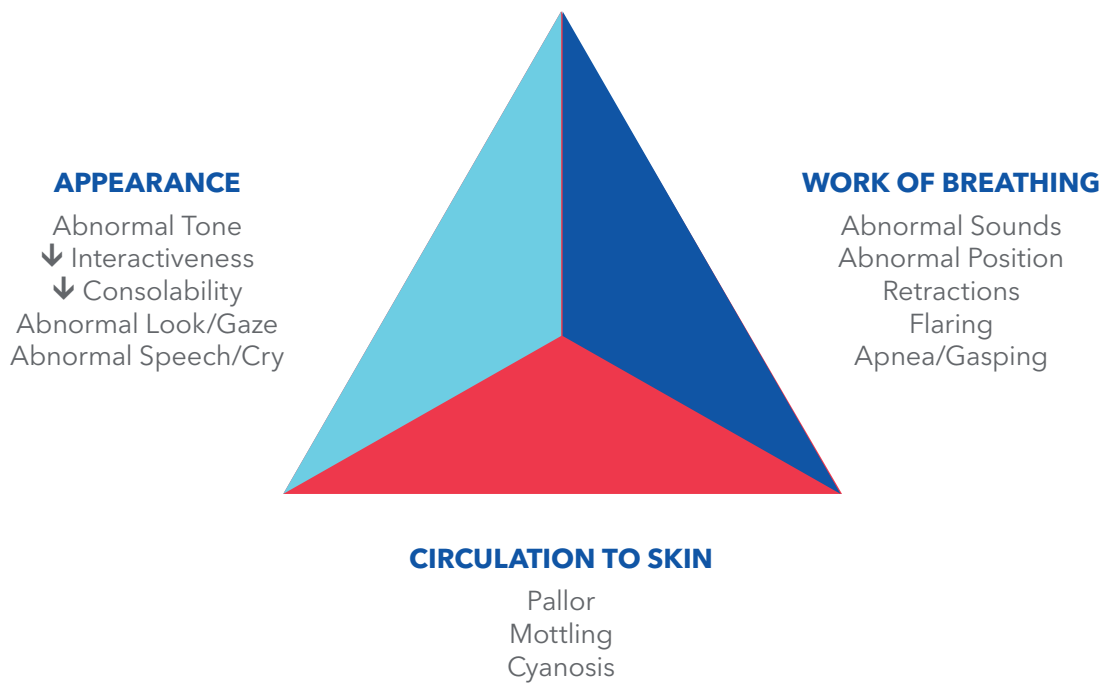


INFANT FEVER/HYPOTHERMIA PROTOCOL (29-60 DAYS OLD)



INFANT FEVER/HYPOTHERMIA PROTOCOL (29-60 DAYS OLD)

PEDIATRIC ASSESSMENT TRIANGLE*:



DEFINITIONS:

IBI: +BCx or +CSF Cx

SBI: IBI or +UCx or +Stool Cx

possible SBI = equivocal UCx or pneumonia with -BCx

NOTES:

1. Note that the focal infections outlined in the exclusion criteria place patients off of the particular pathway. However, we caution the provider that they must all be treated aggressively, which will typically include obtaining cultures and starting appropriate antibiotics.

2. We do not recommend the routine use of viral respiratory panel testing in children of this age group, unless it will

change decision-making decisions. One caveat to this is the consideration to test all febrile infants for Influenza during flu season. Another caveat would be to perform viral testing if needed for cohorting patients in inpatient hospital units.

3. For infants ≤ 28 days old, perform full septic work-up (including LP), initiate IV antibiotics, and admit (please refer to febrile/hypothermic neonate protocol)

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